# Leading a session: Difficult nights



For people trained in psychological first aid, with a supervisor and known referral routes.

**Before the talk, read the facilitator pages from page 3: the words, your limits and**

> **Immediate danger?** Pause the session, stay with the person and get urgent help now. Do not leave them alone. Stay until a health worker or responder takes over; feeling calmer is not the end point. Full procedure and contacts: page 2.




Poster A3




Leaflet, tri-fold

## Your materials

**Poster · a shared focus**
For a group: open the discussion, invite observations and return to the main message. Make sure everyone can see; do not read every line aloud.

**Leaflet · detail to take home**
For a question or a closer look, and to keep or share. Open only the relevant section and offer to read it together.

**Use one or both, in any order. Focus on what people need, not on finishing both materials.**

## Before people arrive

- ☐ Hold the talk in daylight, not just before sleep, in a safe, quiet and accessible place, with the organiser's permission. Hang the poster where all can see it; bring enough leaflets.
- ☐ Fill in and check the help contacts on page 2; do not hand out leaflets with unverified contacts. If there is still a risk, check the local warning system and where to go if a warning comes.
- ☐ Plan language and access: simple words, large print, more time, or a sign language interpreter if needed and available. Offer alternatives if the picture or text is not accessible.

## The session

about 15 minutes · do not rush someone or delay urgent help to finish the plan

**Aim:** each person knows when to ask for help and picks one thing to try tonight; the group names one change for nights here.

### 1 Welcome and explain the purpose

2 min · either material

DO Introduce yourself. Joining is voluntary; nobody has to speak or talk about their nights or what happened.

SAY *"Welcome. Today we will talk about sleep and nights after a distressing experience, and what can make nights a little easier."*

### 2 Look at the picture together

3 min · poster if useful

DO Point to Milan and his father, then to the room around them. Allow silence and different answers. With the leaflet only, use its cover.

ASK *"What is happening in this picture?"*

SAY *"After something very upsetting, many people sleep badly or wake up afraid. Others react differently. You do not have to tell us what happened to ask for support."*

### 3 Explore what helps at night

5 min · move between materials as needed

DO Go through "Before night" and "If you wake up afraid". For each tip, check together whether it is possible here: light, noise, space, privacy. Breathing is optional; stop if it makes someone feel worse.

ASK *"What makes it easier to rest here at night? What could we change together?"*

### 4 Make the route to help clear

3 min · relevant help section

DO Use the help section of either material. Explain the locally checked contact and how to reach help. If there is still a risk, share the local warning plan you checked. Offer a private conversation; use page 2 if concern arises.

SAY *"If poor sleep goes on and makes your days hard, you can ask for help. You do not have to wait."*

ASK *Only if there is still a risk: "If a warning came tonight, what would you do?"*

### 5 Check understanding and close

2 min · offer the leaflet to keep

DO Offer the leaflet and show "Children at night" and "When to ask for help". Agree who takes the group's suggestion to the shelter manager. Thank people and say where to find you.

ASK *"Which one thing will you try tonight?"*

# Safety, referral and worker boundaries

## Get urgent help now if someone:

**SIGNS**

- Talks about suicide, or about harming themselves or someone else
- Cannot keep themselves safe, or cannot care for a child in their care
- Seems very confused, or not aware of where they are

**WHAT TO DO**

1. Stay with the person. Do not leave them alone.
2. Get urgent help now: a health worker, responder or the emergency service in this area.
3. It is okay to ask directly about suicide. Asking does not put the idea in their mind.
4. Stay until a health worker or responder takes over. Feeling calmer is not the end point.

## If someone becomes upset

- Stay calm and stay close.
- Offer a quieter place away from the group, if they wish.
- Do not force them to talk. Ask what they need now.
- Stay as long as they want, or until someone they trust is with them.

**PFA: Look, Listen and Link**

**Look:** safety, urgent needs, people who are very distressed
**Listen:** ask about needs and concerns; help them feel calm
**Link:** basic needs, services, information, loved ones

## Talk privately and link to further help if

- Feelings get worse, do not improve, or make daily life very hard
- They are not able to look after themselves or their children
- A body problem worries them: pain, fever, injury, or not eating or sleeping for days

With the person's agreement: explain the options, go with them if you can, and share only what is needed.

### Do

- Use simple words and respect silence
- Check that each tip is possible here first
- Raise practical problems (light to the toilet, noise, space) with the shelter manager
- Follow up privately with anyone who asks

### Do not

- Promise that the danger will not return, that people are safe, or that sleep will soon be normal
- Give or suggest sleeping medicine
- Ask anyone to describe nightmares or what happened in front of the group
- Ask anyone whether their experience was “trauma”
- Tell people their fear is silly

## Your role

This guide supports a session; it does not replace psychological first aid training. Use your training, supervisor and known referral routes.

Look after yourself too: take breaks, talk with your supervisor or a colleague, and ask for support when you need it.



## Help contacts for this area fill in and check before the talk

**Health worker or facility** _____

**Urgent help** _____

**Other support** _____



Sources: WHO, Psychological First Aid: Guide for Field Workers (2011); WHO, Doing What Matters in Times of Stress (2020); IASC, Guidelines on MHPSS in Emergency Settings (2007); WHO/UNHCR, mhGAP Humanitarian Intervention Guide (2015). This material draws on these publications; it was not created by WHO or IASC, which are not responsible for its content.

# What this material is about

This chapter is about sleep problems after distressing experiences, including events that may be traumatic. The poster and leaflet say "distressing experience" so that people can use the information and ask for support without first deciding whether what happened "counts" as trauma. This is a choice of plain words: it does not mean the experience was not traumatic, and it is not a diagnosis.

## Word key

| ON THE POSTER OR LEAFLET | WHAT WE MEAN | HOW TO HANDLE IT IN THE TALK |
|---|---|---|
| **"A distressing experience"** | Very upsetting experiences, including potentially traumatic events | Do not introduce "trauma". If participants use the word, accept it without correcting them or offering a diagnosis, and ask what they mean (see below) |
| **"Sleep badly or wake up afraid"; "bad dreams"; "lie awake on alert"** | Sleep problems, nightmares and heightened alertness at night as stress reactions | Not PTSD or any other condition on their own. Early help if they continue, get worse or affect daily life |
| **"Notice the safe things around you"** | A grounding technique | Offer it as an option; stop if it makes someone feel worse |
| **"If there is still a risk, check local warnings"** | Realistic safety planning, not reassurance | Never say that people are safe or that the danger will not return |

## Common reactions, and when to look further

**First days and weeks**

Trouble sleeping, nightmares and being on alert at night are common after a distressing experience. They are stress reactions, not a diagnosis. For sleep problems in the first month, WHO recommends relaxation techniques and advice on sleep habits.

**About one month after the event**

If sleep has not improved at all, is getting worse, or tiredness makes daily life hard, link the person to a health worker. Nightmares or reliving the event that continue beyond one month need assessment by a health worker.

**At any time**

Get help now if there is danger (page 2). Offer early help whenever someone is struggling or asks for support: nobody has to wait a month.

## Keep in mind

- People react in different ways. Sleep problems on their own do not mean PTSD or any other condition.
- Nobody has to describe what happened to get support. Focus on what the person needs now, their safety, the support they prefer and how to reach help.
- Keep three levels apart: everyday support, asking for help early, and getting help now.
- Do not promise that sleep will return to normal, or say how long it will take.

## For reference only

**Chronic insomnia** (ICD-11, 7A00): persistent difficulty falling or staying asleep, despite adequate opportunity for sleep, with daytime impairment, at least several times a week for at least three months; shorter episodes are classed as short-term insomnia (7A01).

**Sleeping medicine:** WHO advises that benzodiazepines should not be offered to reduce acute stress symptoms or sleep problems in the first month after a potentially traumatic event. Facilitators never suggest sleeping pills; they link the person to a health worker.

Do not use these terms with participants, and do not diagnose.

**If someone says "trauma" · example: practice cards, page 4**

Do not correct the word and do not start a discussion about trauma. Ask what the person means: "When you say trauma, what do you mean? What do you notice in yourself?" People often use the word for a specific fear or change, for example "I am afraid to go back home because the flood might come again" or "because an earthquake might come". Such fear is a common reaction after a frightening or potentially traumatic experience. Respond to what the person describes, and use the early-help route if it continues, gets worse or makes daily life hard.

Sources: WHO, Psychological First Aid: Guide for Field Workers (2011); WHO, Guidelines for the management of conditions specifically related to stress (2013); WHO, mhGAP module: Assessment and management of conditions specifically related to stress (2013); WHO, ICD-11, 7A00 Chronic insomnia and 7A01 Short-term insomnia; American Psychiatric Association, DSM-5-TR (2022).

# Practice cards

Examples of what people may say and what you can do, step by step. They take place in a private conversation, not in front of the group. The words are an illustration: use your own, follow the person's lead, and use page 2 at once if there is any urgent sign.

WHEN SOMEONE SAYS "TRAUMA"

## "It is my trauma. I wake up at every sound."

From a word to a concrete change you can respond to (page 3, "If someone says 'trauma'").

**1 Ask what they mean**

ASK — *"When you say trauma, what do you mean? What happens at night?"*

THEY SAY — "When someone moves or a door bangs, I jump up. I think the water is coming again."

**2 Name the reaction as common**

SAY — *"After what you went through, being on alert at night is a common reaction. Many people sleep badly or wake up afraid."*

**3 Find what helps them settle**

ASK — *"When you wake up like that, what helps you settle, even a little?"*

DO — Go through "If you wake up afraid" on the leaflet: notice where you are, breathe slowly, go to someone you trust.

**4 Check the warning plan, only if there is still a risk**

DO — Make sure they know the local warning plan and where to go. Never say that the danger will not return.

**5 Keep the route to help open**

SAY — *"If poor sleep goes on and makes your days hard, you can ask for help. You do not have to wait."*

# Practice cards (continued)

WHEN SOMEONE ASKS FOR SLEEPING PILLS

## "Can you get me sleeping pills? I just need to sleep."

Do not give or suggest medicine; link to a health worker (page 2, Do not; page 3).

**1 Be clear about your role**

SAY *"I cannot give or suggest medicine, but I can help you see a health worker about your sleep."*

DO Explain the locally checked contact and offer to go with them.

**2 Listen to what a bad night is like**

ASK *"What usually happens on a bad night?"*

**3 Share one or two leaflet tips for tonight**

DO For example keeping a torch and water close, or a familiar habit before sleep. Mention gently that alcohol or drugs to sleep can make sleep worse.

**4 If they already take medicine from a doctor**

SAY *"Please do not stop it without asking a health worker."*

WHEN A PARENT IS WORRIED ABOUT A CHILD AT NIGHT

## "My son has started wetting the bed again, and he will not sleep without me."

Leaflet, "Children at night".

**1 Name it as common**

SAY *"Many children react like this after a frightening time. Wanting to stay close and wetting the bed again are common."*

**2 Share the leaflet tips**

DO Let the child stay close. Keep a short bedtime routine, such as a story or a song. Tell them simply what the family will do if a warning comes. Do not punish a child for waking up or wetting the bed.

**3 Ask about the parent too**

ASK *"And how are your own nights?"*

**4 Keep the route to help open**

DO Offer a health worker if it goes on and makes days hard, or the parent feels very worried about it.